

إسم المريض رابعياً : منى حلمى عبدالجيد على

الحى / المدينة : الحيزه

المحافظة : الحيزه

رقم البطاقة : 27409022100341 رقم التلفون : 01118658712

العمر (تاريخ الميلاد): 02-09-1974 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة
[11129]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع:

التوقيع:

التوقع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / ۲ ----- / ۲ ----- / ۲. | ----- / ۲. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جـهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: منى حلمى عبدالجيد علي National ID: 27409022100341	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



مبنى
حنس عبدالجيد على
٢٢ ش الشيخ ضرور - المنية
الجزيرة - الجيزة

٢٧٤٠٩٠٢٢١٠٠٣٤١

KS3025752



٢٠١١/٠٨

٢٧٤٠٩٠٢٢١٠٠٣٤١

رقم منزل

المنطقة

البلدية مديرية حنس ٢٠٣٧/٠٩/٣

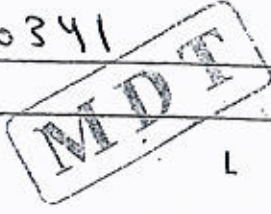




MDT FORM 27/9 / 2025 G

Name	منى حلى عباي	MRN	1216 / 23
Age	51	ID	
Menopausal status	Post Menop	Phone	0111 8658 712
NRS SCORE		Marital status	married & 3 offsprings
BMI		Family history	NO
CO morbidity	HTN	REASON OF MDT	

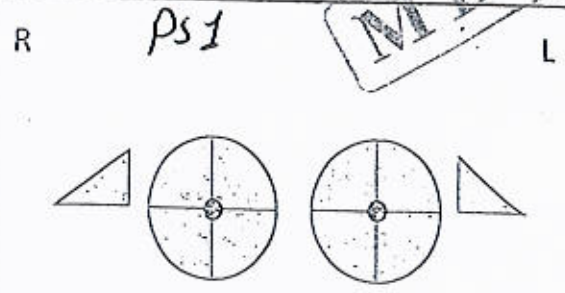
27409022100341



Clinical Data

BX: IDC

ER: 718 PR: 518 HER 2: -ve Ki67: 10%



Sonomammography:

Mets Workup:

De novo metastatic breast cancer to bone (4/2023)TM

AI + Zoladex + Zometa till 12/2023 ⇒ D.P ⇒ Ibrance, fastlocker,

Zometa + Zoladex till 5/2024 ⇒ clinically D.P

Recommendations: ⇒ newly developed bony lesions ⇒

received Taxol weekly, Zometa for 8 weeks

was stationary disease but (G IV toxicity) + skin toxicity, kept on Zoladex, Zoladex

from (9/2024) ⇒ (9/2025) ⇒ CTs (4/2025) :- stationary course.

Bone Scan (5/2025) ⇒ Stationary

MDT OUTCOME & PLAN (But clinical progression) ←

photograph of cancer shift to metastatic spine.

for CT scan with fast

Heavily stat Zoladex (fast)

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name : الأستاذة / منى حلمي عبد الحميد	Request No. : 13002243
Patient No. : 27409022100341	Request Date : 23/03/2023
Path No. : 919/2023	Verification Date : 29/03/2023
Sex : Female	Financial Category : مبادره - اورام دار السلام
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 22/03/2023, 03:15 pm.

Gross Examination:

- 4 grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: No.
- * Angioinvasion: No.

Diagnosis:

Left breast swelling, Tru-cut biopsy:

- Invasive duct carcinoma, GII with focal DCIS.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lobna Omar
Dr. Menna Nabil

Verified by :

Printing Date : 01/04/2023 12:11:04

Page 1 of 1

Printing By : hermal



Presidential Program for Women's Health
Ministry of Health and Population

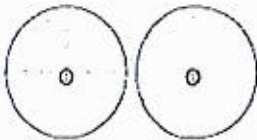
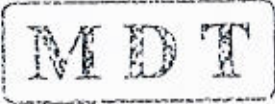
البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID: 1- Name : **منى حلمي عبدالجيد علي** Sex: Female 3- Medical number: 63135496 Old Med : 1216-2023

2-ID number: Patient Age : 50 Y



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE	17/09/2024	

Clinical data

Breast Imaging :			Mets Workup : bone scan:newly developed lesion		
BX IDC	ER 7/8	PR 5/8	Her 2 Neu 1	SISH	Ki67 10
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT:	
T 4b			N 2		
			M 1 bone		

Sonomammography
LT RETROAREALOR 7X4MM+AXLNS 3X1.5CM+SUPRACLAV

PATIENT HISTORY
4/2023-->LT BREAST+AX-MED LNS+BONE METS
5/2023-->STARTED AI/ZOLADEX/ZOMETA
CTS SCAN-->PR OF BREAST LESION
CT CAP - bone scan : PD
12/2023:ibrance+faslodex+zoladex+zometa
5/2024:subjective DP of lt breast + UOQ
compared to bone scan newly developed pelvic & femoral bone lesion
OE : marked clinical progression of left breast mass
patient shifted to taxol weekly
severe toxicity from cth in form of peripheral neuropathy G 3 + skin toxicity
9/2024 CT : SD & PT REPORTS CLINICAL RESPONSE

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
MAINTENANCE (ZOLADEX-NOLVADEX)
ZOMETA

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. شريف حسانين د. يحيى اسماعيل	RADIOLOGY	RADIOLOGY د. شريف حسانين
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY Ultrasound [BOSTON ULTRASOUND]

Patient Name: Mona Helmy Abdel Gayed Ali	Date : 22/03/2023
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 1006	Pat. ID: 30999

**Combined bilateral breast mammography and complementary
ultrasound**

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibroglandular parenchyma density (ACR B)
- Left edema pattern (increased skin thickening and coarse trabeculation)
- Left breast retro areolar spiculated irregular high dense mass lesion , measuring about 116x109mm (in 3D tomosynthesis).
- Left UOQ area of focal asymmetry.
- No right suspicious mass lesion.

Complementary ultrasound :

- left increased skin thickening reaching about 8mm with dilated subdermal lymphatics
- left breast retro areolar spiculated irregular heterogenous hypoechoic solid mass lesion with highly internal vascularity seen within. It infiltrating nipple areolar complex, roughly measuring about 70 x40mm.
- left area from 12 o'clock to 2 o'clock altered pathological parenchyma seen,
- No right cystic or solid mass lesions.
- Normal right skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Right nonspecific axillary lymph nodes.
- Multiple left pathological axillary enlarged LNs with effaced fatty hilum seen at level I and II , III (more than 3 nodes), one at level I with Q extra capsular invasion., the largest one seen at level I measuring about 31x15mm.
- Multiple pathological supra-clavicular node LNs.
- One pathological internal mammary lymph node.

Opinion:

- left retro areolar locally advanced malignant mass lesion with ipsilateral pathological axillary level I, II, III and supra clavicular lymph nodes (BIRADS 5). for histopathological correlation.
- Left pathological internal mammary lymph node
- Right breast normal study BIRADS 1

Signature Dr. Nehal Hamdy

Patient Name	: الأستاذ / مني حلمي عبد الحميد	Request No.	: 13002243
Patient No.	: 27409022100341	Request Date	: 23/03/2023
Path No.	: 919/2023	Verification Date	: 29/03/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 48 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 22/03/2023, 03:15 pm.

Gross Examination:

- 4 grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: No.
- * Angioinvasion: No.

Diagnosis:

Left breast swelling, Tru-cut biopsy:

- Invasive duct carcinoma, GII with focal DCIS.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Lobna Omar
Dr. Menna Nabii

Verified by :

Patient Name : الأستاذ / مني حلمي عبد الحميد	Request No. : 13002243
Patient No. : 27409022100341	Request Date : 23/03/2023
Path No. : 919/2023	Verification Date : 29/03/2023
Sex : Female	Financial Category : أورام دار السلام - مياضه
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 22/03/2023, 03:15 pm.

Gross Examination:

- 4 grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: No.
- * Angioinvasion: No.

Diagnosis:

Left breast swelling, Tru-cut biopsy:

- Invasive duct carcinoma, GII with focal DCIS.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Lobna Omar
Dr. Menna Nabil

Verified by :

Printing Date : 01/04/2023 12:11:04

Page 1 of 1

Printing By : hermal

مستشفى
الرياض
الرياضمبادرة رئيس الجمهورية
لحماية صحة المرأة المصريةالهيئة العامة
لصحة
المرأة

08/30/2026

MDT FORM

Name:	Monna Hilmi Abd Elayed Ali	MRN:	4711
Age:	52 Y	ID:	Z/409022100341
Menopausal Sta	Pre	Phone:*	01156287665
NRS SCORE:		Marital Status:	Divorced
BMI:		Family History:	-VE
Gender		Reason Of MDT:	PD
CO Morbidity:	HTN	ECOG Performance Status	0 0 1 2 3 4
Surgical History:*	-VE		

Clinical Data:

BX: Left Breast IDC G2
ER: 7/8 PR: 5/8 HER2: -ve KI67: 10



T N M

History:

A case of MBC from the start to bone since 4/2023 received AI/Zoladex/Zometa till 12/2023 > PD (PFS 8m)> Ibrance/ Faslodex/ Zoladex/Zometa till 5/2024 > PD (PFS 4 m), received Taxol weekly 8 weeks but developed G4 Skin Toxicity shifted to Nolvadex/ Zoladex from 9/2024 till 9/2025> PD (PFS 1 yr) > Clinical progression although Stationary CTs. On Xeloda+ Zometa from 9/2025 till now but was off tt from 5 till 7/2026. Reports clinical progression of left fungating breast and newly developed inflamed right breast. CTs 6/2026 SD. Bone scan 7/2026 compared to 1/2026 New New skeletal lesions at the lower thoracic spine, mid lumbar spine and bilateral pubic bone

Sonomammography:

Biomarker/Molecular

Metz Workup:

CTs 6/2026 SD. Bone scan 7/2026 compared to 1/2026 New New skeletal lesions at the lower thoracic spine, mid lumbar spine and bilateral pubic bone

Recommendations:

New CTs, TMs, Affinitor vs Chemo.

MDT Outcome & Plan

PORT-CTH
BASELINE CTs AND TM
PALLIATIVE RTH ON BONY LESION PAINFUL + _ FUNGATING BREAST
ZOMETA / 3 MONTHS
START GEM/CAR8 3 MONTHS THEN EVALUTION

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

مستشفى
الرياض
الرياضمبادرة رئيس الجمهورية
لحماية صحة المرأة المصريةالهيئة العامة
لصحة
المرأة

08/30/2026

MDT FORM

Name:	Monna Hilmi Abd Elayed Ali	MRN:	4711
Age:	52 Y	ID:	Z/409022100341
Menopausal Sta	Pre	Phone:*	01156287665
NRS SCORE:		Marital Status:	Divorced
BMI:		Family History:	-VE
Gender		Reason Of MDT:	PD
CO Morbidity:	HTN	ECOG Performance Status	0 0 1 2 3 4
Surgical History:*	-VE		

Clinical Data:

BX: Left Breast IDC G2
ER: 7/8 PR: 5/8 HER2: -ve KI67: 10



T N M

History:

A case of MBC from the start to bone since 4/2023 received AI/Zoladex/Zometa till 12/2023 > PD (PFS 8m)> Ibrance/ Faslodex/ Zoladex/Zometa till 5/2024 > PD (PFS 4 m), received Taxol weekly 8 weeks but developed G4 Skin Toxicity shifted to Nolvadex/ Zoladex from 9/2024 till 9/2025> PD (PFS 1 yr) > Clinical progression although Stationary CTs. On Xeloda+ Zometa from 9/2025 till now but was off tt from 5 till 7/2026. Reports clinical progression of left fungating breast and newly developed inflamed right breast. CTs 6/2026 SD. Bone scan 7/2026 compared to 1/2026 New New skeletal lesions at the lower thoracic spine, mid lumbar spine and bilateral pubic bone

Sonomammography:

Biomarker/Molecular

Metz Workup:

CTs 6/2026 SD. Bone scan 7/2026 compared to 1/2026 New New skeletal lesions at the lower thoracic spine, mid lumbar spine and bilateral pubic bone

Recommendations:

New CTs, TMs, Affinitor vs Chemo.

MDT Outcome & Plan

PORT-CTH
BASELINE CTs AND TM
PALLIATIVE RTH ON BONY LESION PAINFUL + _ FUNGATING BREAST
ZOMETA / 3 MONTHS
START GEM/CAR8 3 MONTHS THEN EVALUTION

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY